

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Study to Evaluate BMS-986278 in Participants With Normal Renal Function, Severe Renal Impairment, or End-Stage Renal Disease on Intermittent Hemodialysis **Short Title/Acronym:** IM027-1014 / BMS-986278 Pharmacokinetics and Safety in Renal Impairment **Protocol Number:** IM027-1014 **NCT Number:** NCT06723535 **Sponsor Name:** Bristol-Myers Squibb **Investigational Product:** BMS-986278 (Admilparant - LPA1 Antagonist) **Phase of Development:** Phase 1 **Medical Monitor:** Bristol-Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the effect of severe renal impairment (RI) and end-stage renal disease (ESRD) with intermittent hemodialysis (IHD) on the pharmacokinetics and safety of a single dose of BMS-986278. **Secondary Objectives:** To evaluate the safety, tolerability, and adverse event profile of a single dose of BMS-986278 across varying degrees of kidney function, including healthy participants with normal renal function.

2.2 Background & Rationale BMS-986278 is an investigational, orally active lysophosphatidic acid receptor 1 (LPA1) antagonist currently being studied in clinical trials for fibrotic conditions such as progressive pulmonary fibrosis. Because patients requiring this therapy may present with comorbid kidney conditions, it is critical to evaluate how severe renal impairment and end-stage renal disease on dialysis impact the drug's absorption, distribution, metabolism, and excretion. This study provides the pharmacokinetic data needed to determine if dose adjustments are required for patients with severely reduced kidney function.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Pharmacokinetic Study) **Control Type:** Active Control / Healthy Volunteer Cohort (Normal Renal Function group) **Blinding:** Open-label **Randomization:** N/A (Staged design based on Renal Impairment severity)

3.2 Planned Enrollment & Duration Target \$N\$: Not explicitly published (Standard Phase 1 PK study sizing, estimated 30-50 participants across cohorts) **Number of Sites:** Multicenter

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Body mass index (BMI) between 18 and 45 kg/m2 inclusive, and body weight ≥ 50 kg at screening (Note: for ESRD participants, body weight should be the prescription dry weight). 2. **Severe RI Cohort:** eGFR < 30 mL/min, not requiring dialysis at screening; medically stable for ≥ 1 month. Supine SBP 90-180 mmHg, DBP 60-100 mmHg, and heart rate 40-110 bpm. 3. **ESRD Cohort:** eGFR < 15 mL/min at screening; on intermittent hemodialysis (average of 3 treatments/week) with adequate dialysis measurements (Kt/V ≥ 1.2 or URR $\geq 65\%$) within the 3 months prior to screening. 4. **Normal Renal Function Cohort:** eGFR ≥ 90 mL/min at screening; Supine SBP 90-160 mmHg, DBP 50-100 mmHg, and heart rate 40-100 bpm.

4.2 Exclusion Criteria 1. Participant with any significant medical condition, laboratory abnormality, or psychiatric illness that may interfere with the study. 2. Any surgical or medical condition(s) possibly affecting drug absorption, distribution, metabolism, or excretion (e.g., bariatric procedure or cholecystectomy). 3. Individuals who are of childbearing potential, breastfeeding, or currently pregnant.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Single-dose administration **Route of Administration:** Oral **Duration of Treatment:** 1 day (single dose), followed by an observation and safety follow-up period of 4 weeks.

5.2 Concomitant & Prohibited Therapy Permitted: Intermittent hemodialysis for the ESRD cohort; standard stable medications for underlying conditions. **Prohibited:** Any therapies, including previous surgeries like bariatric procedures, that strictly conflict with drug absorption, distribution, metabolism, or excretion.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Vitals, BMI, eGFR/Kt/V Labs
Baseline	Day 1	Confinement, Single Dose Administration, Initial PK Blood Draws	
Interim	Days 2-7	Intensive PK Sampling, Safety Labs, ECG, Vital Signs Monitoring	
End of Study	Week 4	Final Safety Follow-up, Exit Assessment, AE/SAE Review	

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Pharmacokinetic parameters of BMS-986278 (including plasma clearance, volume of distribution, C_{max} , AUC , and half-life). **Measurement Method:** Serial blood sampling and central bioanalytical laboratory analysis.

7.2 Secondary & Exploratory Endpoints Incidence and severity of Adverse Events (AEs) and Serious Adverse Events (SAEs). Changes from baseline in vital signs, Electrocardiogram (ECG) parameters, and standard laboratory safety tests.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via physical examinations, vital signs, clinical laboratory tests, and ECGs during the confinement and follow-up period. **Serious Adverse Events (SAEs):** Reported within 24 hours to the sponsor and Institutional Review Board (IRB). **Data Monitoring Committee (DMC):** Internal safety monitoring mechanisms managed by Bristol-Myers Squibb.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Pharmacokinetic (PK) Analysis Set for primary outcomes; Safety Set for all participants receiving at least one dose of the investigational product. **Sample Size Justification:** Sized adequately for a standard Phase 1 PK evaluation to detect clinically meaningful variations in drug exposure between renal cohorts. **Handling of Missing Data:** Standard non-compartmental analysis (NCA) conventions; concentrations below the lower limit of quantification (LLOQ) treated as zero for PK calculations.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Continuous onsite monitoring during the confinement period; source data verification for PK sampling times and accurate AE recording. **Audit Trail:** eCRF locking, tracking of protocol deviations, and data clarification forms via standard quality audit mechanisms.

11. Study Identifiers & Governance

Primary ID: IM027-1014 **Registry Number:** NCT06723535 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** A Study to

Evaluate BMS-986278 in Participants With Normal Renal Function, Severe Renal Impairment, or End-Stage Renal Disease on Intermittent Hemodialysis **Official Regulatory Title:** An Open-Label, Single-Dose, Phase 1 Study to Evaluate the Pharmacokinetics and Safety of BMS-986278 in Participants With Normal Renal Function, Participants With Severe Renal Impairment, and Participants With End-Stage Renal Disease on Intermittent Hemodialysis.

12. Clinical Framework (Renal Impairment Specific)

Primary Condition: Severe Renal Impairment (RI) and End-Stage Renal Disease (ESRD) **Diagnosis Threshold:** $\text{SeGFR} < 30 \text{ mL/min}$ (Severe RI) or $\text{SeGFR} < 15 \text{ mL/min}$ requiring dialysis (ESRD) **Medical Methodology:** Pharmacokinetic (PK) and Safety Evaluation **Optimization Target:** Drug exposure characterization and potential dose adjustment guidance for target populations

13. Study Procedures & Methodology

Management Pathway: Staged cohort enrollment based on renal impairment severity **Education Protocol (HCP):** Site initiation visits and Phase 1 unit procedural training **Education Protocol (Patient):** Informed consent process detailing the confinement period, PK blood draw frequency, and specific dialysis scheduling **Quality Audit Mechanism:** Strict monitoring of exact dosing times and PK blood collection timestamps

14. Observation & Follow-Up Schedule

Total Duration: 4 Follow-up weeks **Onsite Visit Cadence:** Confinement period during Days 1-7, followed by End of Study visit at Week 4 **Remote Monitoring Frequency:** N/A (Primarily onsite Phase 1 evaluation) **Checklist Review Interval:** Daily during the active confinement and dosing period

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				